

Report for P D(32Y/M)

Tests asked Aarogyam Full Body Checkup With Vitamins - New, Pot + 1 Others

Test date 09 May 2025

Report status Complete Report



## 6<sup>STEP</sup> quality control to ensure 100% report accuracy



Qualified and trained technicians



Temperature-controlled containers to store samples



Strict quality checks on samples before processing



Regular monitoring of lab analyzers by experts



Assured machine inspection on a daily basis



Verified reports by qualified pathologists



25+ Years of Trust & Experience



NABL Accredited Labs



100+ Crore Samples Processed

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,POT,ESR

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

## Report Availability Summary

**Note:** Please refer to the table below for status of your tests.

✓ 17 Ready
🟡 0 Ready with Cancellation
🔄 0 Processing
✗ 0 Cancelled in Lab

### TEST DETAILS

### REPORT STATUS

|                                                |         |
|------------------------------------------------|---------|
| POTASSIUM                                      | Ready ✓ |
| ERYTHROCYTE SEDIMENTATION RATE (ESR)           | Ready ✓ |
| AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW | Ready ✓ |
| FASTING BLOOD SUGAR(GLUCOSE)                   | Ready ✓ |
| CHLORIDE                                       | Ready ✓ |
| HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)   | Ready ✓ |
| Lipoprotein (a) [Lp(a)]                        | Ready ✓ |
| SODIUM                                         | Ready ✓ |
| COMPLETE URINE ANALYSIS                        | Ready ✓ |
| HBA PROFILE                                    | Ready ✓ |
| HEMOGRAM - 6 PART (DIFF)                       | Ready ✓ |
| LIVER FUNCTION TESTS                           | Ready ✓ |
| IRON DEFICIENCY PROFILE                        | Ready ✓ |
| KIDPRO                                         | Ready ✓ |
| LIPID PROFILE                                  | Ready ✓ |
| T3-T4-USTSH                                    | Ready ✓ |
| VITAMIN D TOTAL AND B12 COMBO                  | Ready ✓ |
| APOLIPROTEIN RATIO                             | Ready ✓ |

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME      | TECHNOLOGY | VALUE | UNITS |
|----------------|------------|-------|-------|
| HbA1c - (HPLC) | H.P.L.C    | 4.8   | %     |

**Bio. Ref. Interval. :**

**Bio. Ref. Interval.: As per ADA Guidelines**

Below 5.7% : Normal  
 5.7% - 6.4% : Prediabetic  
 >=6.5% : Diabetic

**Guidance For Known Diabetics**

Below 6.5% : Good Control  
 6.5% - 7% : Fair Control  
 7.0% - 8% : Unsatisfactory Control  
 >8% : Poor Control

**Method :** Fully Automated H.P.L.C method

|                             |            |    |       |
|-----------------------------|------------|----|-------|
| AVERAGE BLOOD GLUCOSE (ABG) | CALCULATED | 91 | mg/dL |
|-----------------------------|------------|----|-------|

**Bio. Ref. Interval. :**

90 - 120 mg/dl : Good Control  
 121 - 150 mg/dl : Fair Control  
 151 - 180 mg/dl : Unsatisfactory Control  
 > 180 mg/dl : Poor Control

**Method :** Derived from HBA1c values

**Please correlate with clinical conditions.**

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 12:57  
**Report Released on (RRT)** : 09 May 2025 16:01  
**Sample Type** : EDTA Whole Blood  
**Labcode** : 0905041021/PE005  
**Barcode** : DJ795734




Dr Shibasis MD (Biochemist)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
 NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                            | TECHNOLOGY          | VALUE | UNITS   |
|--------------------------------------|---------------------|-------|---------|
| ERYTHROCYTE SEDIMENTATION RATE (ESR) | MODIFIED WESTERGREN | 3     | mm / hr |

**Bio. Ref. Interval. :-**

Male : 0-15 Female : 0-20

**Clinical Significance:**

- An erythrocyte sedimentation rate (ESR) is a blood test that can rise if you have inflammation in your body. Its also used as a marker to monitor prognosis of an existing inflammatory/infective condition.
- Inflammation is your immune systems response to injury, infection, and many types of conditions, including immune system disorders, certain cancers and blood disorders.
- A high ESR test result may be from a condition that causes inflammation, such as: Arteritis, Arthritis, Systemic vasculitis, Polymyalgia rheumatica, Inflammatory bowel disease, Kidney disease, Infections like Tuberculosis etc, Rheumatoid arthritis and other autoimmune diseases, Heart disease, Certain cancers and many other Conditions.
- A low ESR test result may be caused by conditions such as: A blood disorder, such as: Polycythemia, Sickle cell disease (SCD), Leukocytosis, Heart failure, Certain kidney and liver problems etc.
- Certain physiological conditions also affect ESR results, these include : Pregnancy, menstrual cycle, ageing, obesity, drinking alcohol regularly, and exercise, Certain medicines and supplements also can affect ESR results.
- Hence Its always suggested to interpret ESR results in conjunction with Clinical History and other findings.

**References :**

<https://medlineplus.gov/lab-tests/erythrocyte-sedimentation-rate-esr/>

**Please correlate with clinical conditions.**

**Method:-** MODIFIED WESTERGREN

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 12:57  
**Report Released on (RRT)** : 09 May 2025 16:01  
**Sample Type** : EDTA Whole Blood  
**Labcode** : 0905041021/PE005  
**Barcode** : DJ795734



Dr Arpita MD (Path)

**NAME :** P D(32Y/M)  
**REF. BY :** SELF  
**TEST ASKED :** AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                                  | METHODOLOGY           | VALUE              | UNITS                        | Bio. Ref. Interval. |
|--------------------------------------------|-----------------------|--------------------|------------------------------|---------------------|
| <b>HEMOGLOBIN</b>                          | SLS-Hemoglobin Method | 14.2               | g/dL                         | 13.0-17.0           |
| Hematocrit (PCV)                           | CPH Detection         | 44.1               | %                            | 40.0-50.0           |
| Total RBC                                  | HF & EI               | 5.39               | X 10 <sup>6</sup> /μL        | 4.5-5.5             |
| <b>Mean Corpuscular Volume (MCV)</b>       | <b>Calculated</b>     | <b><u>81.8</u></b> | <b>fL</b>                    | <b>83.0-101.0</b>   |
| <b>Mean Corpuscular Hemoglobin (MCH)</b>   | <b>Calculated</b>     | <b><u>26.3</u></b> | <b>pq</b>                    | <b>27.0-32.0</b>    |
| Mean Corp.Hemo. Conc (MCHC)                | Calculated            | 32.2               | g/dL                         | 31.5-34.5           |
| Red Cell Distribution Width - SD (RDW-SD)  | Calculated            | 39.3               | fL                           | 39-46               |
| Red Cell Distribution Width (RDW - CV)     | Calculated            | 13.2               | %                            | 11.6-14             |
| RED CELL DISTRIBUTION WIDTH INDEX (RDWI)   | Calculated            | 200.3              | -                            | *Refer Note below   |
| MENTZER INDEX                              | Calculated            | 15.2               | -                            | *Refer Note below   |
| <b>TOTAL LEUCOCYTE COUNT (WBC)</b>         | HF & FC               | 6.05               | X 10 <sup>3</sup> / μL       | 4.0 - 10.0          |
| <b>DIFFERENTIAL LEUCOCYTE COUNT</b>        |                       |                    |                              |                     |
| Neutrophils Percentage                     | Flow Cytometry        | 55.5               | %                            | 40-80               |
| Lymphocytes Percentage                     | Flow Cytometry        | 39.3               | %                            | 20-40               |
| Monocytes Percentage                       | Flow Cytometry        | 2.3                | %                            | 2-10                |
| Eosinophils Percentage                     | Flow Cytometry        | 2.3                | %                            | 1-6                 |
| Basophils Percentage                       | Flow Cytometry        | 0.3                | %                            | 0-2                 |
| Immature Granulocyte Percentage (IG%)      | Flow Cytometry        | 0.3                | %                            | 0-0.5               |
| Nucleated Red Blood Cells %                | Flow Cytometry        | 0.01               | %                            | 0.0-5.0             |
| <b>ABSOLUTE LEUCOCYTE COUNT</b>            |                       |                    |                              |                     |
| Neutrophils - Absolute Count               | Calculated            | 3.36               | X 10 <sup>3</sup> / μL       | 2.0-7.0             |
| Lymphocytes - Absolute Count               | Calculated            | 2.38               | X 10 <sup>3</sup> / μL       | 1.0-3.0             |
| <b>Monocytes - Absolute Count</b>          | <b>Calculated</b>     | <b><u>0.14</u></b> | <b>X 10<sup>3</sup> / μL</b> | <b>0.2 - 1.0</b>    |
| Basophils - Absolute Count                 | Calculated            | 0.02               | X 10 <sup>3</sup> / μL       | 0.02 - 0.1          |
| Eosinophils - Absolute Count               | Calculated            | 0.14               | X 10 <sup>3</sup> / μL       | 0.02 - 0.5          |
| Immature Granulocytes (IG)                 | Calculated            | 0.02               | X 10 <sup>3</sup> / μL       | 0-0.3               |
| Nucleated Red Blood Cells                  | Calculated            | 0.01               | X 10 <sup>3</sup> / μL       | 0.0-0.5             |
| <b>PLATELET COUNT</b>                      | HF & EI               | 280                | X 10 <sup>3</sup> / μL       | 150-410             |
| <b>Mean Platelet Volume (MPV)</b>          | <b>Calculated</b>     | <b><u>12.4</u></b> | <b>fL</b>                    | <b>6.5-12</b>       |
| <b>Platelet Distribution Width (PDW)</b>   | <b>Calculated</b>     | <b><u>16.7</u></b> | <b>fL</b>                    | <b>9.6-15.2</b>     |
| <b>Platelet to Large Cell Ratio (PLCR)</b> | <b>Calculated</b>     | <b><u>45.3</u></b> | <b>%</b>                     | <b>19.7-42.4</b>    |
| Plateletcrit (PCT)                         | Calculated            | 0.35               | %                            | 0.19-0.39           |

**Remarks :** Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

**\*Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

**Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)**

**(Reference : \*FC- flowcytometry, \*HF- hydrodynamic focussing, \*EI- Electric Impedence, \*Hb- hemoglobin, \*CPH- Cumulative pulse height)**

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 12:57  
**Report Released on (RRT)** : 09 May 2025 16:01  
**Sample Type** : EDTA Whole Blood  
**Labcode** : 0905041021/PE005  
**Barcode** : DJ795734

  
 Dr Arpita MD (Path)

Page : 3 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
 NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                             | METHODOLOGY          | VALUE       | UNITS     | Bio. Ref. Interval. |
|---------------------------------------|----------------------|-------------|-----------|---------------------|
| <b>Complete Urinogram</b>             |                      |             |           |                     |
| <b><u>Physical Examination</u></b>    |                      |             |           |                     |
| VOLUME                                | Visual Determination | 3           | mL        | -                   |
| COLOUR                                | Visual Determination | PALE YELLOW | -         | Pale Yellow         |
| APPEARANCE                            | Visual Determination | CLEAR       | -         | Clear               |
| SPECIFIC GRAVITY                      | pKa change           | 1.01        | -         | 1.003-1.030         |
| PH                                    | pH indicator         | 6.5         | -         | 5-8                 |
| <b><u>Chemical Examination</u></b>    |                      |             |           |                     |
| URINARY PROTEIN                       | PEI                  | ABSENT      | mg/dL     | Absent              |
| URINARY GLUCOSE                       | GOD-POD              | ABSENT      | mg/dL     | Absent              |
| URINE KETONE                          | Nitroprusside        | ABSENT      | mg/dL     | Absent              |
| URINARY BILIRUBIN                     | Diazo coupling       | ABSENT      | mg/dL     | Absent              |
| UROBILINOGEN                          | Diazo coupling       | Normal      | mg/dL     | <=0.2               |
| BILE SALT                             | Hays sulphur         | ABSENT      | -         | Absent              |
| BILE PIGMENT                          | Ehrlich reaction     | ABSENT      | -         | Absent              |
| URINE BLOOD                           | Peroxidase reaction  | ABSENT      | -         | Absent              |
| NITRITE                               | Diazo coupling       | ABSENT      | -         | Absent              |
| LEUCOCYTE ESTERASE                    | Esterase reaction    | ABSENT      | -         | Absent              |
| <b><u>Microscopic Examination</u></b> |                      |             |           |                     |
| MUCUS                                 | Microscopy           | ABSENT      | -         | Absent              |
| RED BLOOD CELLS                       | Microscopy           | ABSENT      | cells/HPF | 0-5                 |
| URINARY LEUCOCYTES (PUS CELLS)        | Microscopy           | ABSENT      | cells/HPF | 0-5                 |
| EPITHELIAL CELLS                      | Microscopy           | ABSENT      | cells/HPF | 0-5                 |
| CASTS                                 | Microscopy           | ABSENT      | -         | Absent              |
| CRYSTALS                              | Microscopy           | ABSENT      | -         | Absent              |
| BACTERIA                              | Microscopy           | ABSENT      | -         | Absent              |
| YEAST                                 | Microscopy           | ABSENT      | -         | Absent              |
| PARASITE                              | Microscopy           | ABSENT      | -         | Absent              |

(Reference : \*PEI - Protein error of indicator, \*GOD-POD - Glucose oxidase-peroxidase)

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:23  
**Report Released on (RRT)** : 09 May 2025 17:04  
**Sample Type** : URINE  
**Labcode** : 0905076851/PE005  
**Barcode** : DK610690

  
 Dr Arpita MD (Path)

Page : 4 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
 NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                    | TECHNOLOGY | VALUE | UNITS |
|------------------------------|------------|-------|-------|
| FASTING BLOOD SUGAR(GLUCOSE) | PHOTOMETRY | 94    | mg/dL |

**Bio. Ref. Interval. :-**

| As per ADA Guideline: Fasting Plasma Glucose (FPG) |                        |
|----------------------------------------------------|------------------------|
| <b>Normal</b>                                      | 70 to 100 mg/dl        |
| <b>Prediabetes</b>                                 | 100 mg/dl to 125 mg/dl |
| <b>Diabetes</b>                                    | 126 mg/dl or higher    |

Note :

The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

**Please correlate with clinical conditions.**

**Method:-** GOD-PAP METHOD

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:39  
**Report Released on (RRT)** : 09 May 2025 15:29  
**Sample Type** : FLUORIDE PLASMA  
**Labcode** : 0905078018/PE005  
**Barcode** : DT169835



Dr Shibasis MD (Biochemist)

Page : 5 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME | TECHNOLOGY | VALUE | UNITS |
|-----------|------------|-------|-------|
|-----------|------------|-------|-------|

|                                |                  |                    |              |
|--------------------------------|------------------|--------------------|--------------|
| <b>25-OH VITAMIN D (TOTAL)</b> | <b>E.C.L.I.A</b> | <b><u>18.6</u></b> | <b>ng/mL</b> |
|--------------------------------|------------------|--------------------|--------------|

**Bio. Ref. Interval. :**

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml  
 Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

**Clinical Significance:**

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.

Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.

Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%

Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1)87-98.

**Method :** Fully Automated Electrochemiluminescence Competitive Immunoassay

|              |           |     |       |
|--------------|-----------|-----|-------|
| VITAMIN B-12 | E.C.L.I.A | 337 | pg/mL |
|--------------|-----------|-----|-------|

**Bio. Ref. Interval. :**

Normal: 197-771 pg/ml

**Clinical significance :**

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

**Method :** Fully Automated Electrochemiluminescence Competitive Immunoassay

**Please correlate with clinical conditions.**

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                    | TECHNOLOGY         | VALUE | UNITS |
|------------------------------|--------------------|-------|-------|
| APOLIPOPROTEIN - A1 (APO-A1) | IMMUNOTURBIDIMETRY | 124   | mg/dL |

**Bio. Ref. Interval. :**

Male : 86 - 152

Female : 94 - 162

**Method :** FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

|                            |                    |    |       |
|----------------------------|--------------------|----|-------|
| APOLIPOPROTEIN - B (APO-B) | IMMUNOTURBIDIMETRY | 74 | mg/dL |
|----------------------------|--------------------|----|-------|

**Bio. Ref. Interval. :**

Male : 56 - 145

Female : 53 - 138

**Method :** FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

|                                 |            |     |       |
|---------------------------------|------------|-----|-------|
| APO B / APO A1 RATIO (APO B/A1) | CALCULATED | 0.6 | Ratio |
|---------------------------------|------------|-----|-------|

**Bio. Ref. Interval. :**

Male : 0.40 - 1.26

Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

**Method :** Derived from serum Apo A1 and Apo B values

**Please correlate with clinical conditions.**

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
BASANTA PLAZA LAXMI NARAYAN COLONY  
NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                     | TECHNOLOGY         | VALUE | UNITS |
|-------------------------------|--------------------|-------|-------|
| Lipoprotein (a) [Lp(a)]       | IMMUNOTURBIDIMETRY | 2.92  | mg/dL |
| <b>Bio. Ref. Interval. :-</b> |                    |       |       |

Adults : < 30.0 mg/dl

Clinical Significance:

Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:

Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:

Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

**Please correlate with clinical conditions.**

**Method:-** LATEX ENHANCED IMMUNOTURBIDIMETRY

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

Page : 8 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
 NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                                    | TECHNOLOGY         | VALUE | UNITS |
|----------------------------------------------|--------------------|-------|-------|
| HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP) | IMMUNOTURBIDIMETRY | 0.24  | mg/L  |

**Bio. Ref. Interval. :-**

< 1.00 - Low Risk  
 1.00 - 3.00 - Average Risk  
 >3.00 - 10.00 - High Risk  
 > 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

**Clinical significance:**

High sensitivity C- reactive Protein ( HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

**Kit Validation Reference:**

- 1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
- 2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

**Please correlate with clinical conditions.**

**Method:-** FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                                                                                                                         | TECHNOLOGY | VALUE | UNITS |
|-----------------------------------------------------------------------------------------------------------------------------------|------------|-------|-------|
| IRON                                                                                                                              | PHOTOMETRY | 119.1 | µg/dL |
| <b>Bio. Ref. Interval. :</b><br>Male : 65 - 175<br>Female : 50 - 170<br><b>Method :</b> Ferrozine method without deproteinization |            |       |       |
| TOTAL IRON BINDING CAPACITY (TIBC)                                                                                                | PHOTOMETRY | 327.9 | µg/dL |
| <b>Bio. Ref. Interval. :</b><br>Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl<br><b>Method :</b> Spectrophotometric Assay         |            |       |       |
| % TRANSFERRIN SATURATION                                                                                                          | CALCULATED | 36.32 | %     |
| <b>Bio. Ref. Interval. :</b><br>13 - 45<br><b>Method :</b> Derived from IRON and TIBC values                                      |            |       |       |
| UNSAT.IRON-BINDING CAPACITY(UIBC)                                                                                                 | PHOTOMETRY | 208.8 | µg/dL |
| <b>Bio. Ref. Interval. :</b><br>162 - 368<br><b>Method :</b> SPECTROPHOTOMETRIC ASSAY                                             |            |       |       |
| <b>Please correlate with clinical conditions.</b>                                                                                 |            |       |       |

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGRAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY NABAPALLY  
 BARASAT NAVAPALLY BIRATI

| TEST NAME                       | TECHNOLOGY        | VALUE             | UNITS        | Bio. Ref. Interval. |
|---------------------------------|-------------------|-------------------|--------------|---------------------|
| TOTAL CHOLESTEROL               | PHOTOMETRY        | 157               | mg/dL        | < 200               |
| HDL CHOLESTEROL - DIRECT        | PHOTOMETRY        | 47                | mg/dL        | 40-60               |
| HDL / LDL RATIO                 | CALCULATED        | 0.46              | Ratio        | > 0.40              |
| <b>LDL CHOLESTEROL - DIRECT</b> | <b>PHOTOMETRY</b> | <b><u>103</u></b> | <b>mg/dL</b> | <b>&lt; 100</b>     |
| TRIG / HDL RATIO                | CALCULATED        | 1.79              | Ratio        | < 3.12              |
| TRIGLYCERIDES                   | PHOTOMETRY        | 84                | mg/dL        | < 150               |
| TC/ HDL CHOLESTEROL RATIO       | CALCULATED        | 3.4               | Ratio        | 3 - 5               |
| LDL / HDL RATIO                 | CALCULATED        | 2.2               | Ratio        | 1.5-3.5             |
| VLDL CHOLESTEROL                | CALCULATED        | 16.89             | mg/dL        | 5 - 40              |
| NON-HDL CHOLESTEROL             | CALCULATED        | 110.36            | mg/dL        | < 160               |

**Please correlate with clinical conditions.**

**Method :**

CHOL - Cholesterol Oxidase, Esterase, Peroxidase  
 HCHO - Direct Enzymatic Colorimetric  
 HD/LD - Derived from HDL and LDL values.  
 LDL - Direct Measure  
 TRI/H - Derived from TRIG and HDL Values  
 TRIG - Enzymatic, End Point  
 TC/H - Derived from serum Cholesterol and Hdl values  
 LDL/ - Derived from serum HDL and LDL Values  
 VLDL - Derived from serum Triglyceride values  
 NHDL - Derived from serum Cholesterol and HDL values

**\*REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

| TOTAL CHOLESTEROL | (mg/dl) | HDL  | (mg/dl) | LDL             | (mg/dl) | TRIGLYCERIDES   | (mg/dl) |
|-------------------|---------|------|---------|-----------------|---------|-----------------|---------|
| DESIRABLE         | <200    | LOW  | <40     | OPTIMAL         | <100    | NORMAL          | <150    |
| BORDERLINE HIGH   | 200-239 | HIGH | >60     | NEAR OPTIMAL    | 100-129 | BORDERLINE HIGH | 150-199 |
| HIGH              | >240    |      |         | BORDERLINE HIGH | 130-159 | HIGH            | 200-499 |
|                   |         |      |         | HIGH            | 160-189 | VERY HIGH       | >500    |
|                   |         |      |         | VERY HIGH       | >190    |                 |         |

**Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.**

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

Page : 11 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY NABAPALLY  
 BARASAT NAVAPALLY BIRATI

| TEST NAME                                 | TECHNOLOGY        | VALUE               | UNITS      | Bio. Ref. Interval. |
|-------------------------------------------|-------------------|---------------------|------------|---------------------|
| ALKALINE PHOSPHATASE                      | PHOTOMETRY        | 95.4                | U/L        | 45-129              |
| BILIRUBIN - TOTAL                         | PHOTOMETRY        | 0.98                | mg/dL      | 0.3-1.2             |
| BILIRUBIN -DIRECT                         | PHOTOMETRY        | 0.17                | mg/dL      | < 0.3               |
| BILIRUBIN (INDIRECT)                      | CALCULATED        | 0.81                | mg/dL      | 0-0.9               |
| GAMMA GLUTAMYL TRANSFERASE (GGT)          | PHOTOMETRY        | 21.91               | U/L        | < 55                |
| SGOT / SGPT RATIO                         | CALCULATED        | 0.67                | Ratio      | < 2                 |
| <b>ASPARTATE AMINOTRANSFERASE (SGOT )</b> | <b>PHOTOMETRY</b> | <b><u>38.87</u></b> | <b>U/L</b> | <b>&lt; 35</b>      |
| <b>ALANINE TRANSAMINASE (SGPT)</b>        | <b>PHOTOMETRY</b> | <b><u>57.82</u></b> | <b>U/L</b> | <b>&lt; 45</b>      |
| PROTEIN - TOTAL                           | PHOTOMETRY        | 7.15                | gm/dL      | 5.7-8.2             |
| ALBUMIN - SERUM                           | PHOTOMETRY        | 4.21                | gm/dL      | 3.2-4.8             |
| SERUM GLOBULIN                            | CALCULATED        | 2.94                | gm/dL      | 2.5-3.4             |
| SERUM ALB/GLOBULIN RATIO                  | CALCULATED        | 1.43                | Ratio      | 0.9 - 2             |

**Please correlate with clinical conditions.**

**Method :**

ALKP - Modified IFCC method  
 BILT - Vanadate Oxidation  
 BILD - Vanadate Oxidation  
 BILI - Derived from serum Total and Direct Bilirubin values  
 GGT - Modified IFCC method  
 OT/PT - Derived from SGOT and SGPT values.  
 SGOT - IFCC\* Without Pyridoxal Phosphate Activation  
 SGPT - IFCC\* Without Pyridoxal Phosphate Activation  
 PROT - Biuret Method  
 SALB - Albumin Bcg<sup>1</sup>method (Colorimetric Assay Endpoint)  
 SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES  
 A/GR - Derived from serum Albumin and Protein values

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

Page : 12 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
BASANTA PLAZA LAXMI NARAYAN COLONY  
NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME | TECHNOLOGY       | VALUE | UNITS  |
|-----------|------------------|-------|--------|
| POTASSIUM | I.S.E - INDIRECT | 4.74  | mmol/L |

**Bio. Ref. Interval. :**

ADULTS: 3.5-5.1 MMOL/L

**Clinical Significance :**

An abnormal increase in potassium (hyperkalemia) can profoundly affect the nervous system and increase the chance of irregular heartbeats (arrhythmias), which, when extreme, can be fatal. The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed, icteric or lipemic. The concentration of Potassium in a given specimen may vary due to differences in assay methods, calibration and reagent specificity.

**Method :** ION SELECTIVE ELECTRODE - INDIRECT

**Please correlate with clinical conditions.**

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS - NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY NABAPALLY  
 BARASAT NAVAPALLY BIRATI

| TEST NAME                  | TECHNOLOGY       | VALUE  | UNITS  | Bio. Ref. Interval. |
|----------------------------|------------------|--------|--------|---------------------|
| BLOOD UREA NITROGEN (BUN)  | PHOTOMETRY       | 10.06  | mg/dL  | 7.94 - 20.07        |
| UREA (CALCULATED)          | CALCULATED       | 21.53  | mg/dL  | Adult : 17-43       |
| CREATININE - SERUM         | PHOTOMETRY       | 0.87   | mg/dL  | 0.72-1.18           |
| UREA / SR.CREATININE RATIO | CALCULATED       | 24.75  | Ratio  | < 52                |
| BUN / SR.CREATININE RATIO  | CALCULATED       | 11.56  | Ratio  | 9:1-23:1            |
| CALCIUM                    | PHOTOMETRY       | 9.46   | mg/dL  | 8.8-10.6            |
| URIC ACID                  | PHOTOMETRY       | 5.72   | mg/dL  | 4.2 - 7.3           |
| SODIUM                     | I.S.E - INDIRECT | 137.02 | mmol/L | 136 - 145           |
| CHLORIDE                   | I.S.E - INDIRECT | 103.89 | mmol/L | 98 - 107            |

**Please correlate with clinical conditions.**

**Method :**

BUN - Kinetic UV Assay.  
 UREAC - Derived from BUN Value.  
 SCRE - Creatinine Enzymatic Method  
 UR/CR - Derived from UREA and Sr.Creatinine values.  
 B/CR - Derived from serum Bun and Creatinine values  
 CALC - Arsenazo III Method, End Point.  
 URIC - Uricase / Peroxidase Method  
 SOD - ION SELECTIVE ELECTRODE - INDIRECT  
 CHL - ION SELECTIVE ELECTRODE - INDIRECT

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961



Dr Shibasis MD (Biochemist)

Page : 14 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
 NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY NABAPALLY  
 BARASAT NAVAPALLY BIRATI

| TEST NAME                          | TECHNOLOGY       | VALUE            | UNITS        | Bio. Ref. Interval. |
|------------------------------------|------------------|------------------|--------------|---------------------|
| <b>TOTAL TRIIODOTHYRONINE (T3)</b> | <b>E.C.L.I.A</b> | <b><u>78</u></b> | <b>ng/dL</b> | <b>80-200</b>       |
| TOTAL THYROXINE (T4)               | E.C.L.I.A        | 7.33             | µg/dL        | 4.8-12.7            |
| TSH - ULTRASENSITIVE               | E.C.L.I.A        | 1.87             | µIU/mL       | 0.54-5.30           |

**Comments :** IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

**The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.**

**Method :**

T3 - Fully Automated Electrochemiluminescence Compititive Immunoassay  
 T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay  
 USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

**Disclaimer :**

Results should always be interpreted using the reference range provided by the laboratory that performed the test.  
 Different laboratories do tests using different technologies, methods and using different reagents which may cause difference  
 In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.  
 To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961

  
 Dr Shibasis MD (Biochemist)

Page : 15 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

**NAME** : P D(32Y/M)  
**REF. BY** : SELF  
**TEST ASKED** : AAROGYAM FULL BODY CHECKUP WITH VITAMINS -  
 NEW,ESR,POTASSIUM

**HOME COLLECTION :**  
 BASANTA PLAZA LAXMI NARAYAN COLONY  
 NABAPALLY BARASAT NAVAPALLY BIRATI

| TEST NAME                              | TECHNOLOGY | VALUE | UNITS          |
|----------------------------------------|------------|-------|----------------|
| EST. GLOMERULAR FILTRATION RATE (eGFR) | CALCULATED | 118   | mL/min/1.73 m2 |

**Bio. Ref. Interval. :-**

> = 90 : Normal  
 60 - 89 : Mild Decrease  
 45 - 59 : Mild to Moderate Decrease  
 30 - 44 : Moderate to Severe Decrease  
 15 - 29 : Severe Decrease

**Clinical Significance**

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

**Reference**

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

**Please correlate with clinical conditions.**

**Method:-** 2021 CKD EPI Creatinine Equation

~~ End of report ~~

**Sample Collected on (SCT)** : 09 May 2025 08:18  
**Sample Received on (SRT)** : 09 May 2025 13:38  
**Report Released on (RRT)** : 09 May 2025 16:22  
**Sample Type** : SERUM  
**Labcode** : 0905077982/PE005  
**Barcode** : DL328961




Dr Shibasis MD (Biochemist)

Page : 16 of 17

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

#### CONDITIONS OF REPORTING

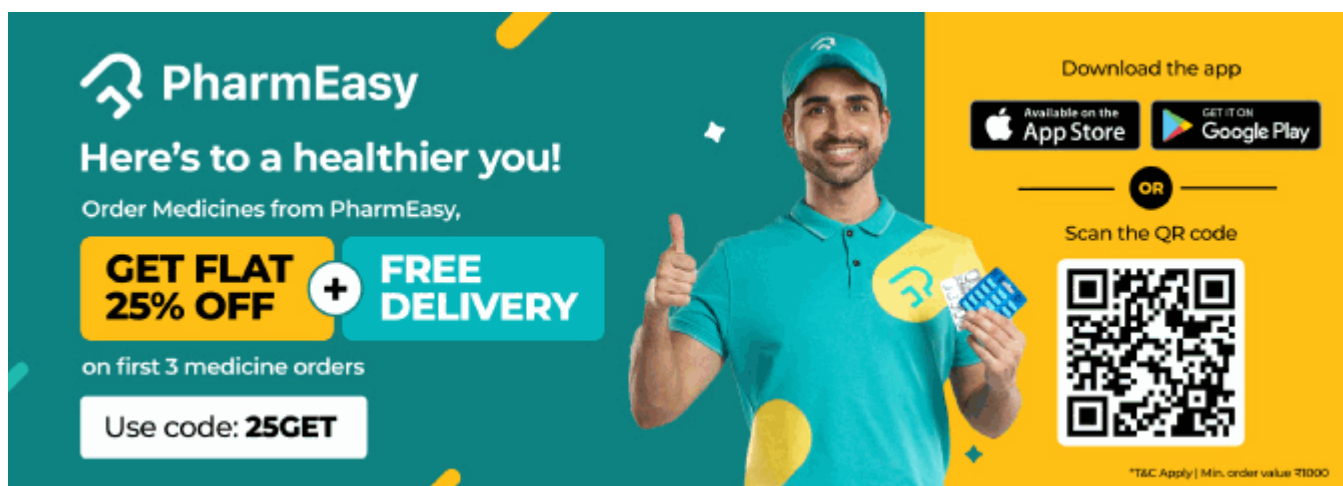
- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Docon Technologies Private Limited, Thyrocare Technologies Limited and its employees/representatives do not assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

#### EXPLANATIONS

- ✓ **Name** - The name is as declared by the client and recorded by the personnel who collected the specimen.
- ✓ **Ref.By** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- ✓ **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- ✓ **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- ✓ **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

#### SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ For suggestions, complaints or feedback, write to us at [grievance-office@docon.co.in](mailto:grievance-office@docon.co.in) or call us on 7022000900.



The advertisement banner for PharmEasy features a smiling male delivery person in a blue uniform and cap, giving a thumbs up and holding a medicine box. The background is split into teal and yellow sections. On the teal side, the PharmEasy logo is at the top, followed by the slogan 'Here's to a healthier you!' and 'Order Medicines from PharmEasy,'. A large yellow button highlights 'GET FLAT 25% OFF + FREE DELIVERY' on first 3 medicine orders, with a code box below showing 'Use code: 25GET'. On the yellow side, it says 'Download the app' with 'Available on the App Store' and 'GET IT ON Google Play' buttons. Below these is a QR code with the text 'Scan the QR code' and a small disclaimer at the bottom right: '\*T&C Apply | Min. order value ₹1000'.